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Pharmacy Law & Ethics (Phar 4243)

Group Assignment

Group 4

**ETHIOPIAN PHARMACEUTICAL PHARMACOVIGILANCE LAW AND
SYSTEM IN ETHIOPIA**

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Submitted to: Nathan

Introduction

- Pharmacovigilance is the science and activities related to detecting, assessing, understanding, and preventing adverse effects or any drug-related problems.
- Its important as it protects patients, detects safety issues early, improves rational medicine use and trust, reduces healthcare costs, and supports regulatory decisions.
- PV is coordinated through systems like WHO-Uppsala Monitoring Centre (UMC).
- In Ethiopia PV is regulated by the Ethiopian Food and Drug Authority (EFDA).
- Ethiopia is a full member of the WHO Programme for International Drug Monitoring.



Legal Framework for PV in Ethiopia

- EFDA is the primary regulatory body responsible for ensuring the safety, quality, and efficacy of medicines.
- Hosts the National Pharmacovigilance Centre and coordinates with regional health bureaus, hospitals, and stakeholders.
- EFDA operationalizes PV through several supporting documents, such as:
 - ✓ **Pharmacovigilance Directive** – outlines reporting requirements and responsibilities for facilities and MAHs.
 - ✓ **National Pharmacovigilance Guidelines** – covers systems for ADR reporting, signal detection, and risk-minimization.
 - ✓ **Good Pharmacovigilance Practice (GVP) Guidelines** – adapted from international standards for Ethiopia's context.
 - ✓ **Post-Marketing Surveillance (PMS) Guidelines** – ensure ongoing monitoring of product quality and safety.

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- Under the EFDA Proclamation No. 1112/2019 (and earlier versions), the Authority is legally mandated to:
 - ✓ Establish a national pharmacovigilance system.
 - ✓ Collect and evaluate reports of adverse drug reactions and product complaints.
 - ✓ Regulate safety of medicines throughout their lifecycle.
 - ✓ Enforce reporting obligations for:
 - Marketing authorization holders (MAHs)
 - Health professionals
 - Health facilities

Objective of Ethiopian pharmacovigilance system

Ensuring patient safety and quality of patient care related to the use of medicines.

- Promoting rational medicine use by providing timely, evidence-based information on the safety, quality, and efficacy of medicines.
- Supporting regulatory decisions on medicines, which can range from providing warnings to product withdrawal or recall.
- Monitoring product quality defects, such as counterfeit, falsified, and substandard products
- Improving public health and safety in relation to all medical and paramedical interventions.

Structure of National PV Center (EFDA)

- National PV Center (EFDA) coordinates PV, manages ADR database, performs signal detection, and issues safety alerts.
- Organized under the PV & Clinical Trials Directorate with units for ADR case management, risk assessment, communication/training, and data reporting.
- Regional PV Units collect reports, check quality, train facilities, and monitor regional safety trends.
- Reporting pathway: **Health Facilities → Regional PV Units → National PV Center (EFDA) → WHO-VigiBase.**

Responsibilities & Roles of MAH and HCP

Responsibilities of Marketing Authorization Holders (MAHs)

- MAHs must designate a **Qualified Person Responsible for Pharmacovigilance (QPPV)** who must be resident in Ethiopia to manage their system.
- They are required to document their entire PV framework in a detailed report known as the **Pharmacovigilance System Master File (PSMF)**.
- MAHs must submit **Periodic Safety Update Reports (PSURs)** to the EFDA, summarizing and re-evaluating their product's global safety profile on an ongoing basis.
- They must establish and implement **Risk Management Plans (RMPs)** to outline the necessary activities for preventing or minimizing identified risks.
- A critical duty is **expedited reporting** of serious unexpected adverse reactions (SUSARs) within strictly defined, short timelines.

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Role of Healthcare Professionals (HCPs)

- HCPs have a **mandatory legal and ethical duty** to report all suspected adverse events, including those for products not officially registered or those with an unknown causal link.
- The scope of their reporting extends beyond classic Adverse Drug Reactions (ADRs) to cover **Medication Errors (MEs), lack of efficacy, and product quality defects**.
- Spontaneous reports from HCPs are the backbone of the national system, providing the essential raw data for signal detection and analysis.
- HCPs are crucial in accurately completing the standardized national reporting form (ADE-ADR-ME) and ensuring the inclusion of all necessary details.
- They must also cooperate with the NPC by providing **follow-up information** requested for complete case documentation and accurate causality assessment.

The National Reporting System and Events

- The system operates primarily via **Spontaneous Reporting**, a passive surveillance method where reports are voluntarily submitted by observers (HCPs, public).
- The standardized national form (ADE-ADR-ME) is the official tool used for capturing reports on adverse events related to medicines.
- Specialized protocols and forms are used for monitoring **Adverse Events Following Immunization (AEFIs)**, which are crucial for vaccine safety programs.
- The system covers all types of product safety issues, including problems with the **quality** or **efficacy** of the drug observed in real-world use.
- The current focus is on migrating from manual paper submissions to a more timely and efficient **digital reporting platform** to enhance data processing speed.

Adverse Reaction Reporting Requirements (Expedited)

- **Expedited Reporting** is a strict requirement for all **Serious Unexpected Suspected Adverse Reactions (SUSARs)** identified by MAHs and clinical trial sponsors.
- For events classified as **Fatal or Life-Threatening SUSARs**, the maximum reporting deadline to the EFDA is **7 calendar days** from first awareness.
- For all other **Serious SUSARs**, the reporting period is extended slightly to a mandatory submission within **15 calendar days**.
- These requirements ensure that the regulatory authority receives immediate notification of the most critical and potentially dangerous new safety issues.
- Compliance with these timelines is heavily scrutinized during GVP inspections and is essential for rapid risk assessment and public protection.

Risk Management Tools in Practice

- **Risk Management Plans (RMPs)** are applied to specific products to systematically identify, characterize, and minimize their known and potential risks throughout their lifecycle.
- The **Periodic Safety Update Reports (PSURs)** provide EFDA with a regular, comprehensive summary of the product's safety data for an ongoing benefit-risk re-evaluation.
- The **Signal Management** process involves validating potential signals identified from aggregated reports and determining if regulatory action is warranted.
- These tools facilitate a **proactive, evidence-based approach** to drug safety, moving beyond just reacting to individual reports.
- Effective implementation of RMPs and PSURs is a core measure of an MAH's compliance and commitment to product safety.

Regulatory Actions and Outcomes

- Regulatory decisions are based on the **ongoing assessment** of whether a medicine's benefits continue to outweigh its risks in the Ethiopian context.
- Actions often involve **mandating updates** to the official product labeling, including the Summary of Product Characteristics (SmPC) and patient leaflets.
- EFDA utilizes **Direct Healthcare Professional Communications (DHPCs)** and public warnings to rapidly inform prescribers and the public of new, critical risks.
- More severe measures, if risks are deemed unmanageable, include imposing **usage restrictions, temporary suspension, or permanent market revocation** of the product.
- Transparency is maintained by ensuring that the reasons and evidence supporting all regulatory decisions are effectively communicated to all stakeholders

Major Challenges in the System

- 1. Low Reporting Culture:** The most significant obstacle is the pervasive underreporting of adverse events by HCPs, resulting in a crucial data deficit for safety surveillance.
- 2. Resource Limitations:** Many health institutions and local companies lack the necessary infrastructure, dedicated PV staff, and financial provision for PV activities.
- 3. Knowledge and Attitude Gaps:** A persistent lack of awareness, insufficient training, and negative professional attitudes towards reporting hinder full system functionality.
- 4. Inefficient Infrastructure:** The dependence on slow, manual, and often fragmented paper-based reporting systems delays data processing and signal detection.
- 5. Inconsistent Compliance:** There is a substantial variation in the implementation of GVP standards, particularly among local manufacturers and health facilities.

Current Initiatives and Future Direction

1. **Legal Enhancement:** The introduction of the comprehensive **Pharmacovigilance Directive No. 932/2022** has significantly modernized the legal framework.
2. **Capacity Building:** Ongoing efforts are focused on expanding training, awareness campaigns, and integrating PV education into health curricula.
3. **Digital Focus:** There is a clear strategic shift towards implementing and maximizing the use of **digital reporting systems** to overcome infrastructure challenges.
4. **Regulatory Strengthening:** Future focus includes increased rigor in **GVP inspections and enforcement** to drive compliance among MAHs and institutions.
5. **Goal:** To establish a **resilient, proactive, and data-rich PV infrastructure** essential for safeguarding patient safety in Ethiopia's growing pharmaceutical market.

How to report?

- The reporting of medicine related AEs will be mandatorily written at the end of all prescription papers so that consumers could be reminded that they can use any of the reporting tools to report medicine related harms.

1. Paper based tools (yellow form)
2. Web Based (e-reporting) System
3. Med safety Mobile App
4. Toll free telephone number: 8482
5. Email: pharmacovigilance@efda.gov.et.

Ethiopian Food and Drug Authority (EFDA)
REPORTING FORM FOR ADVERSE EVENTS FOLLOWING IMMUNIZATION (AEFI)

*Patient Name or initials: _____
*Patient's full Address: _____
Telephone: _____
Sex: M ☐ F ☐ Pregnant ☐ Lactating ☐
*Date of birth: ____/____/____
Or Age at onset: ☐ years ☐ months ☐ days
Or Age group at onset: < 1 year ☐ 1 to 5 yrs ☐ 6 to 15 yrs ☐
16 to 60 yrs ☐ >60 yrs ☐

*Reporter's Name: _____
Institution: _____
Title & Department: _____
Address: _____
Telephone: _____
Email: _____
Date patient notified event to health system: ____/____/____
Today's date: ____/____/____

Health facility (place or vaccination center) name & address: _____

Vaccine					
*Name of vaccine	*Brand Name and, Name of Manufacturer	*Date of vaccination	*Time of vaccination	*Dose (1 st , 2 nd , etc)	*Batch /Lot number

Diluents (if applicable)				
Name of diluent	*Batch /Lot number	Expiry date	Date of reconstitution	Time of reconstitution

*Adverse event(s):
☐ Severe local reaction ☐ >3 days ☐ beyond nearest joint
☐ Seizures ☐ febrile ☐ afebrile
☐ Abscess
☐ Septis
☐ Encephalopathy
☐ Toxic shock syndrome
☐ Thrombocytopenia
☐ Anaphylaxis
☐ Fever ≥38°C
☐ Injection site reaction
☐ Generalized itch
☐ Rash

AEFI Onset Date: ____/____/____
AEFI Onset Time: ____
Resolved Date (leave blank if ongoing): ____/____/____
Resolved Time: ____
Describe AEFI (Signs & Symptoms): _____

*Serious: Yes/No: ☐ Yes ☐ No ☐ Death ☐ Life threatening ☐ Persistent or significant disability ☐ Hospitalization
☐ Congenital anomaly ☐ Other important medical event (Specify): _____

*Outcome: ☐ Recovering ☐ Recovered ☐ Recovered with sequelae ☐ Not Recovered ☐ Unknown
☐ Died If died, date of death: ____/____/____ Autopsy done: ☐ Yes ☐ No ☐ Unknown

Past medical history (including history of similar reaction or other allergies), concomitant medication and dates of administration (exclude those used to treat reaction) other relevant information (e.g., other cases), use additional sheet if needed: _____

First decision making level to complete:
Investigation needed: ☐ Yes ☐ No If yes, date investigation planned: ____/____/____

National level to complete:
Date report received at national level: ____/____/____ AEFI worldwide unique ID: _____
Comments: _____

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This ADE reporting form is prepared and printed by Ethiopian Food and Drug Authority (EFDA) in collaboration with the USAID Global Health Supply Chain Program- Procurement and Supply Management (GHSC-PSM) project.

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EFDA
Ethiopian Food and Drug Authority
Ethiopian Food and Drug Authority

Other means of Reporting
Electronic Reporting form on our website: www.efda.gov.et

P.O. Box 5681-Tel.011-523142
Addis Ababa, Ethiopia

The End

Reference

- *Ethiopian Food and Drug Authority (EFDA). National Pharmacovigilance Guideline, 4th Edition. EFDA, 2024. Document No.: EFDA/GDL/006, Version 004. Addis Ababa, Ethiopia. [PDF]*
- *Ethiopian Food and Drug Authority (EFDA). Pharmacovigilance Directive No. 932/2022. EFDA, Addis Ababa, Ethiopia.*
- *World Health Organization (WHO)–Uppsala Monitoring Centre (UMC). International Drug Monitoring: The Role of National Centres. WHO, 2002.*
- *Guideline for Consumer Reporting of Adverse Event of Medicines*
<https://share.google/vfTsmg7S7E3YMiKHt>

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